



Reducing Costs, Avoiding Crises:

A Smarter Model for Serious Illness Care

How health plans can reduce expenditures, lower utilization, and improve member experience and outcomes by identifying the right members, engaging them early, and orchestrating effective home-based palliative care.



Executive Summary



up to
\$3,494
net monthly savings
per patient

Managing members with life-limiting, serious illnesses is a persistent challenge for health plans. When proactive, preventative care is absent, patients are more likely to experience acute episodes that lead to hospitalizations, avoidable costs, and fragmented care experiences. For payers, this creates a significant financial and utilization burden; for members, it often results in unnecessary suffering and a lack of coordinated support.

↓43%
reduction in emergency
department (ED) visits

Vynca's home-based palliative care model addresses these gaps. By blending advanced analytics, risk identification, and an integrated, interdisciplinary care team, Vynca identifies high-risk members earlier, engages them before crises, and delivers coordinated, whole-person support at home—where it matters most.

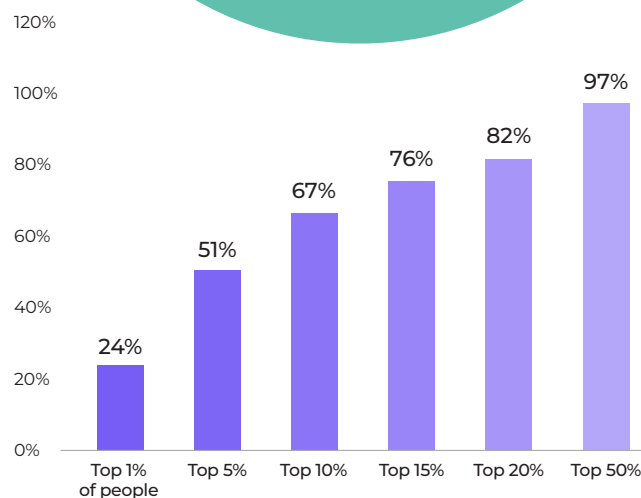
↓52%
reduction in inpatient
admissions

The results are substantial and proven across high-complexity Medicaid and Medicare populations: up to \$3,494 net monthly savings per patient, a 43% reduction in emergency department (ED) visits, a 52% reduction in inpatient admissions, and marked improvements in member satisfaction and clinical outcomes.





1% of members
drive **24%** of
healthcare spend*



The Unmet Need: The High Cost of Unmanaged Serious Illness

While a relatively small segment of health plan members live with serious, life-limiting illnesses, they account for a disproportionate share of healthcare spending. Unlike other patient populations, their needs are complex, often fluctuating between periods of stability and acute decline. Without proactive identification and coordinated management, changes in their condition can go unnoticed until they escalate, leading to missed opportunities for earlier, more effective intervention. These patients require more comprehensive, ongoing support than standard primary or specialty care typically provides, yet many do not meet the strict criteria for hospice—leaving them in a gap where critical needs often go unaddressed.

Life with serious illness happens in this in-between space, resulting in disconnected care that contributes to outsized costs for these individuals. In fact, the consequences of fragmented, poorly coordinated care are reflected in national spending patterns: 1% of the U.S. population—primarily those with complex, serious illnesses in their last year of life—account for more than 24% of all health care expenditures. [\[source\]](#)

Impact on Health Plans

- **Financial Burden:** Unmanaged serious illness drives high rates of avoidable ED visits and inpatient admissions, contributing to unsustainable costs. [\[PubMed\]](#)
- **Utilization Challenges:** Reactive, crisis-driven care leads to inefficient resource allocation and makes it difficult to achieve value-based care goals and quality metrics.
- **Member and Provider Experience:** Members endure unmanaged symptoms and frequent crises; providers lack the tools and support to manage complex, high-need patients.

The Limits of Traditional Approaches

Current protocols struggle to provide continuous, coordinated support for these members. Fragmented care leaves both patients and providers dissatisfied and overwhelmed, with no clear “off-ramp” from the treadmill of escalating interventions.

Health Equity

and Access to Care

Many of the highest-cost and most complex members also face significant inequities that limit their access to timely palliative care and these gaps are major contributors to unmanaged serious illness and poor outcomes. Disparities in access to palliative care are most pronounced among rural communities, low-income populations, and racial and ethnic minorities.

- **Rural communities** are universally recognized as having pronounced access barriers due to distance, workforce shortages, and limited healthcare infrastructure, resulting in significantly lower rates of palliative care access compared to urban areas [\[PMC\]](#).
- **Medicaid populations** experience both financial and structural barriers—including lower reimbursement rates, fewer local providers, and a higher prevalence of unmet social needs. These factors contribute to lower palliative care utilization and poorer end-of-life experiences for Medicaid members, who are among the most vulnerable in the healthcare system [\[PMC\]](#).
- **Racial and ethnic minorities** (including Black, Hispanic/Latino, American Indian/Alaska Native, and Asian American populations) face well-documented disparities in palliative care access, often due to cultural barriers, language differences, mistrust, and provider bias [\[PMC, CAPC\]](#).

These groups face systemic, geographic, and socioeconomic barriers that limit their ability to receive timely, high-quality palliative care—leading to higher symptom burden, avoidable hospitalizations, and poorer outcomes. Addressing these inequities is essential to delivering on the promise of whole-person care for all.



Paradigm Shift:

The Power of Palliative Care

Redefining Palliative Care

Palliative care is often misunderstood as synonymous with hospice. In reality, this approach offers specialized support for those living with serious illness—focused on relief from symptoms, clarity around choices, and improved quality of life, while the patient continues to undergo treatments aimed at curing the disease. Specialized palliative care brings more clarity, comfort, and control to patients and families, supporting them throughout their illness journey.

Engaging patients earlier—before crises—dramatically reduces avoidable hospitalizations and improves the member experience [\[CoalitionCCC\]](#). Home-based models delivered by interdisciplinary teams address the full spectrum of clinical, emotional, social, and practical needs. This approach is especially effective for high-complexity populations, such as those in rural or underserved areas.

Key Components of an Effective Palliative Care Program

Interdisciplinary Team Model

Clinical care, symptom management, medication oversight, mental health counseling, spiritual support, and social services are integrated to address the full range of member needs.

Technology-Enabled Identification and Coordination

Advanced analytics and AI-driven platforms identify high-risk members, optimize outreach and enrollment, and coordinate timely interventions [\[PMC\]](#).

Advance Care Planning (ACP)

Early, structured conversations about goals and preferences ensure care aligns with what matters most to each member.

Dedicated technology platforms are essential for effective patient identification, engagement, and coordination. Predictive analytics and machine learning enable care teams to anticipate needs, intervene early, and prevent crises—ensuring that care finds the patient, not the other way around [\[PMC\]](#).

Value-Based Palliative Care

The case for transforming how palliative care is funded and delivered is clear.

Traditional fee-for-service reimbursement rewards volume, not value. The payment structure doesn't support the comprehensive, interdisciplinary support that people with serious illness require. As a result, palliative care outside of the hospital or hospice in the U.S. has struggled to scale. The payment doesn't incentivize practices to innovate or focus on outpatient care outside of hospice organizations.

Vynca is pioneering a different approach.

We think of reimbursement in a different way. We align to value and put our fees at risk — aligning payment with measurable outcomes in patient satisfaction, quality, outcomes, and avoidable utilization. We are about providing our patients more quality days at home™ while reducing total spend for payers.

Current payment models do not work

For high-need, high-cost populations, the reimbursement on a fee-for-service schedule creates fundamental barriers:

- It does **not** cover the interdisciplinary clinical, social, and behavioral expertise required for whole-person care.
- It promotes reactive, visit-based care, not early identification, proactive engagement, and longitudinal support.
- It provides **no mechanism** to measure — let alone reward — patient experience, quality of life, or cost containment.

The result: fragmented care, late interventions, avoidable hospital use, and missed opportunities to support members earlier in their journey.



What Makes a Model “Value-Based”?

In palliative care, “value” must be defined and measured — not assumed. Vynca’s value-based approach ties performance to three domains:

1. Patient Experience

- Member and family satisfaction
- Feeling supported, informed, and able to navigate their illness

2. Quality Outcomes

- Symptom reduction
- Documented goals of care
- Timely, appropriate hospice transition
- Increased median days in hospice
- Reduced burden on caregivers

3. Cost & Utilization

- Fewer avoidable ED visits
- Reduction in unplanned hospital admissions
- Lower total cost of care for high-need populations

These are outcomes that matter to members and to health plans — and they are the foundation of a scalable, sustainable value-based palliative care model.

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Quality Metrics

Palliative care programs can also positively impact health plan quality metrics—including HEDIS® and Medicare Advantage Star Ratings—by improving clinical outcomes, reducing avoidable utilization, and supporting patient-centered care.

The National Committee for Quality Assurance (NCQA) recognizes that standard quality metrics are not always appropriate for patients with advanced illness. As such, NCQA has implemented exclusions for palliative care patients on fifteen key HEDIS measures so health plans are not penalized when palliative care patients forgo interventions or screenings that do not align with their goals of care. This allows health plans to strategically invest in palliative care programs and focus on clinically relevant, patient-centered outcomes without pressure to pursue inappropriate interventions solely to meet quality metrics.

Advance care planning is now a dedicated HEDIS measure.

HEDIS measures also account for a significant portion of Medicare Advantage Star Ratings. By excluding palliative care patients from specific measures, health plans can improve overall compliance rates, boost Star Ratings, and unlock quality bonus payments from CMS. In addition, palliative care’s demonstrated ability to reduce hospitalizations and ED visits can further improve performance on utilization-based quality measures, supporting both quality and financial objectives.

Furthermore, advance care planning is now a dedicated HEDIS measure. Palliative care teams can facilitate ACP discussions and documentation, driving higher rates of ACP completion than national averages. This not only improves member experience and care alignment but also directly supports quality reporting requirements.

Plans that effectively manage palliative care populations can:

- **Demonstrate** compassionate, appropriate care in their marketing materials
- **Differentiate** themselves in markets among senior populations
- **Enhance** member experience scores through effective palliative care

In summary, integrating palliative care not only improves outcomes for members with serious illness but also strengthens health plan quality performance and aligns with evolving regulatory standards.

The Strategic Opportunity for Health Plans

For government and employer-sponsored plans taking on greater risk for complex populations, value-based palliative care represents a powerful lever to:

- **Improve** member outcomes and quality of life
- **Reduce** avoidable utilization and total cost of care
- **Meet** regulatory requirements tied to quality, population health, and risk management
- **Enhance** member experience, satisfaction, and retention

But success requires partners purpose-built for these models. Vynca's integrated, technology-enabled approach is designed to deliver meaningful value under risk-bearing arrangements — not months from now, but starting on day one.



We bring the clinical model, analytics, and operational rigor needed to make value-based palliative care real — and to deliver compassionate, whole-person support for the members who need it most.

The Vynca Solution: Orchestrating High-Impact, Home-Based Palliative Care

Vynca's mission is to proactively support people living with serious illness through an orchestrated blend of clinical expertise, intelligent technology, and flexible care delivery—ensuring members and families receive exactly what they need, when and where they need it.



We help people with serious illnesses spend more quality days at home™. We help people stay out of the hospital and in control of their lives, at any age and any stage of a serious illness or injury.

Vynca's Differentiated, Tech-Powered Approach

Core Services: Symptom and medication management, crisis response, social and behavioral health support, spiritual counseling, and comprehensive advance care planning.

Hybrid Care Delivery: In-person when it matters, virtual when it works—always coordinated and holistic, with a singular focus on palliative care.

People-Led, Technology-Enabled: Technology enables an intelligent, integrated care experience, allowing the care team to deliver timely, compassionate interventions before problems become crises.



Having the support and resources from Vynca is the reason I am better.
It is the reason I'm taking my health into my own hands.

—Toni, Vynca patient

Data, Predictive Analytics and Proactive Intervention

Palliative care is designed to proactively relieve symptoms and improve quality of life. Yet in many healthcare systems, referrals are triggered late—often after avoidable escalation has already occurred. Vynca advances the palliative care model by using predictive analytics and real-time data to identify rising risk earlier, surface unmet needs, and guide more timely intervention. This proactive approach delivers measurable value for health plans by reducing avoidable utilization and ensuring members receive the right level of support at the right time.

A New Standard: Anticipatory, Not Reactive

Our analytics predict potential future clinical risk—often before deterioration is apparent in claims or clinical documentation. These insights guide earlier visits and proactive touchpoints, allowing care teams to intervene sooner, stabilize members, and reduce the likelihood of avoidable crises.



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Risk Stratification That Aligns Resources to Need

Not all seriously ill members require the same intensity of care. Vynca's risk models stratify members by:

- Likelihood of hospitalization
- Symptom burden trends
- Medication complexity
- Caregiver strain
- Social drivers of health
- Patterns of escalating utilization

This allows care teams to prioritize those who will benefit most from high-touch support—improving efficiency and maximizing return on investment for payer partners.

Insights Beyond Claims

Claims alone cannot reveal what truly drives avoidable utilization. Our platform captures and integrates:

- Functional decline indicators
- Behavioral health needs
- Social instability
- Missed appointments
- Symptom trajectory changes
- Caregiver capacity

This 360° view gives plans a clearer picture of risk and enables more precise resource allocation.

Predictive Monitoring That Prevents Crises

Real-time signals—including symptom reporting, medication issues, vitals, and environmental factors—feed into Vynca's monitoring system. When risk elevates, the care team engages immediately, often preventing ED visits or hospitalizations.

We Don't Just Connect Care - We Orchestrate It

Vynca's hybrid model and intelligent technology platform work behind the scenes to anticipate needs and intervene early. We prevent crises before they happen, reduce avoidable hospitalizations, and help people spend more quality days at home™

-  We prevent crises before they happen
-  We reduce avoidable hospitalizations
-  We help people spend more quality days at home

For plans, this translates to:

- Fewer preventable admissions
- Lower MLR
- Improved quality performance
- Better member experience and continuity

Data That Enhances, Not Replaces, Clinical Judgment

Predictive analytics guide the care model; clinicians personalize it. This hybrid approach ensures that every intervention is both data-informed and human-centered—supporting better outcomes and more quality days at home.

Provider Empowerment & Team Experience

This proactive, tech-enabled model not only benefits members, it also transforms the experience of the clinicians caring for them. Caring for high-need, seriously ill members places significant pressure on physicians and advanced practitioners. Vynca's hybrid care model and technology extend the provider's team, reduce administrative burden, and create the coordinated environment clinicians need to deliver exceptional care.

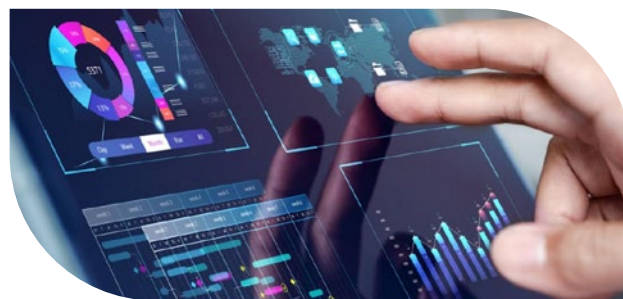
How Vynca Strengthens the Provider Experience

An Extension of the Physician's Team

Vynca's interdisciplinary team—physicians, NPs, nurses, social workers, and care coordinators—works alongside community providers, not in place of them. This shared-care model lightens the load on treating physicians while ensuring members receive continuous, whole-person support.

Technology That Reduces Friction

Our platform automates time-consuming administrative tasks and surfaces real-time, actionable insights. Providers gain visibility into symptoms, social needs, medication issues, and emerging risks—without sifting through fragmented records.



Coordinated, Proactive Care

Predictive analytics and 24/7 monitoring help teams intervene earlier, often preventing avoidable ED visits or hospitalizations. Providers benefit from knowing their patients have support between appointments—and that small issues won't escalate into crises.

Reducing Burnout & Improving Clinical Satisfaction

By absorbing administrative tasks, managing psychosocial complexity, and coordinating care across settings, Vynca decreases cognitive load and emotional strain. Providers report:

- Greater ability to focus on treatment
- Improved work-life balance
- Less stress managing high-acuity cases
- The ability to practice at the top of their license

This strengthened clinical experience directly contributes to better outcomes and more stable provider networks—key priorities for payer partners.

Burnout among clinicians caring for seriously ill populations impacts member outcomes, access, and continuity of care. Vynca's model alleviates the most common pain points by:

- Automating routine administrative tasks
- Coordinating care across home, clinic, and hospital settings
- Managing complex psychosocial needs providers struggle to address alone
- Offering real-time insights that guide timely intervention

By extending the care team and removing friction from clinical workflows, Vynca makes high-quality, person-centered care sustainable—for both members **and** the clinicians who serve them.

Measurable Impact: Vynca's Proven Results

Vynca's home-based palliative care model is designed to address social determinants of health that impact equity and care access for rural, low-income, and other underserved populations—including racial and ethnic minorities. By leveraging technology-enabled outreach, virtual and in-person visits, and a multidisciplinary team that addresses clinical, social, and behavioral needs, Vynca expands access for these underserved populations.

Partnership Health (PHC) observed

\$3,494

in net monthly savings per enrolled member, resulting in a 2.75:1 return on investment (ROI)

In a recent [Medicaid-focused study](#), Vynca's Intensive Outpatient Palliative Care (IOCP) program achieved a 43% reduction in ED visits, a 52% reduction in inpatient admissions, and \$1,700 in monthly per-member savings for Medicaid plans—demonstrating the model's scalability and value for high-need, underserved populations. In internal evaluations, Partnership Health (PHC) observed \$3,494 in net monthly savings per enrolled member, resulting in a 2.75:1 return on investment (ROI). Vynca treated over two-thirds of PHC's enrolled patients, establishing it as the largest palliative care provider in the network.

Vynca's effectiveness extends across payer types, with commercial and other populations seeing similarly strong outcomes, underscoring the effectiveness of Vynca's model.

Financial Impact for Payers

- **\$1,700 - \$3,494** net monthly savings per member
- **3:1 ROI** for health plans

Utilization Impact

- **\$1,43%** reduction in ED visits
- **52%** reduction in inpatient admissions
- **3x longer** hospice stays (median 53 days vs 17 days national average), indicating more appropriate end-of-life planning following Vynca services

Clinical Outcomes & Member Satisfaction

- **\$95%** patient satisfaction score
- **81%** symptom improvement after 6 weeks
- **79%** ACP document completion (vs. 28% nationally)

Partnering with Vynca:

A Strategic Advantage for Health Plans

Vynca is more than a provider—it is a proven, strategic partner committed to transforming care for seriously ill members through an integrated, scalable model built on intelligent technology and flexible care delivery.

Why Partner with Vynca?

- Specialized palliative care delivered via a hybrid model
- Predictive technology and an expert interdisciplinary team focused solely on helping those with serious illness spend more quality days at home™
- A proven track record of reducing costs, improving utilization, and advancing value-based care
- Seamless integration into existing workflows, with implementation support, training, and ongoing performance analytics
- Adaptable to diverse populations—including Medicaid and commercial plans—and tailored for rural, underserved, or high-acuity populations

Vynca's orchestrated, tech-enabled palliative care model represents a smarter, more compassionate approach to serious illness care: delivering measurable value for health plans and providers while addressing the unique needs of each member.



"With my oncologist I felt like I was just a number, on a rotation, but with Vynca I felt heard. My palliative team has taken such good care of me, I can't even tell you. I'm so blessed, it was the best thing that happened to me in 7 years of fighting cancer - finding them."
— Vynca Patient

How to Get Started

Vynca has the ability to identify the right patients at the right time, and can perform an Opportunity Analysis that includes the identification of your members who are most likely to be eligible for palliative care.

To discuss how Vynca can help your plan achieve its cost, utilization, and value-based care goals, [schedule](#) an Opportunity Analysis with Vynca.

Vynca orchestrates the right care at the right time, wherever patients are—because serious illness deserves serious support.

For more information visit vyncacare.com/healthplans, email connect@vyncacare.com.